



Hospital Registration Number

Demographics

Service Name Lab Practice EMS	Service No. 100	CACC/ACS 921	Call Number 2026-0919-0412	Call Date 2026/09/19	
Last Name Martel			First Name Joanne		
Age 58 Y	Sex F	Weight (kg) 70	Date of Birth 1968/01/30	Health Insurance Number 0000-000-000	Version ZZ

Mailing Address

Street No. 17	Street Name Birch Crescent	City/Town Riverside	Province ON	Postal Code K0L 0A0	Country Canada
Pick-up Location or Sending Facility (City/Town) Riverside					Pick-up Code R

Clinical Information

Date of Occurrence 2026/09/19	Time of Occurrence 12:45	Chief Complaint Confusion, known insulin-dependent diabetic	☐ Positive for FREI
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Incident History

Husband found pt at kitchen table, confused and sweaty, at approx 1245. She took her usual insulin at 0730 and skipped lunch while gardening. Last seen normal at approx 1130. No trauma, no seizure activity, no incontinence per husband. No alcohol.

MOHLTC DNR Confirmation Number

	Trauma Location	Problem Type	Site/Type Mechanism
1			
2			
3			

Relevant Past History

Provided by: ☒ Patient ☒ Other: Husband (Paul)

☐ Previously Healthy ☐ Cardiac ☐ Stroke/TIA ☐ Seizure ☐ Psychiatric ☐ Cancer ☐ CNO
☐ Respiratory ☒ Hypertension ☒ Diabetes ☐ Anaphylaxis ☐ Other (list below)

Details Type 1 diabetes since age 19 (insulin), hypertension. Last low glucose episode 2 years ago. Provided by husband, confirmed by pt after treatment.

Medications

☐ None ☐ Nitrates ☒ Insulin/Oral Diabetic Meds ☐ Phosphodiesterase Inhibitors
☐ CNO ☐ ASA ☐ Blood thinner/Anticoagulants ☐ Salbutamol ☐ Furosemide

Other Insulin glargine 20 units every morning (taken 0730), insulin lispro with meals (none since breakfast), perindopril 4 mg daily.

Allergies

☐ NKA ☐ CNO ☒ Other – list below

Details Sulfa drugs (rash)

Treatment Prior to Arrival

☐ None ☐ EFRT ☐ Physician ☐ Fire ☐ Bystander ☐ CNO
☐ Midwife ☐ Other Paramedic ☐ Nurse ☐ Police ☐ Self ☒ Other (list below)

Details Husband tried orange juice at approx 1250. Pt held it in her mouth and coughed, so he stopped.

Cardiac Arrest Information

	Date	Start Time
Arrest Witnessed By ☐ Bystander ☐ Trained Responder ☐ Paramedic ☐ Unwitnessed		
CPR Started By ☐ Bystander ☐ Trained Responder ☐ Paramedic ☐ None		
First Shock By ☐ Bystander ☐ Trained Responder ☐ Paramedic		

Physical Exam

General Appearance Skin Colour Pale Skin Condition Diaphoretic

Seated at kitchen table, eyes open, pale and diaphoretic. Confused to time and place, following simple commands slowly.

Head/Neck Trachea ☒ - Midline Shifted ☐ - R ☐ - L JVD ☐ - Elevated ☒ - Not Elevated

No signs of head injury. No tongue bite.

Chest Air Entry ☒ - Bilaterally Decreased ☐ - R ☐ - L Breath Sounds ☒ - Clear ☐ - Wheezes ☐ - Crackles ☐ - Rub ☐ - Absent

Equal air entry, clear bilaterally.

Abdomen ☒ - Soft ☐ - Rigid ☐ - Distended ☐ - Tender ☐ - Mass ☐ - Pulsatile ☐ - RU ☐ - LU ☐ - LL ☐ - RL ☐ - Center

Soft, non-tender.

Back/Pelvis ☒ - Unremarkable

Extremities ☐ - Unremarkable Peripheral Edema ☒ - Absent ☐ - Present Pedal Pulse ☐ - Absent ☒ - Present

Moves all limbs equally. Strength equal bilaterally.

Practice tools

Check my ACR

Clear highlights

Read back

Review page

Lock for submission

Unlock (instructor)

How this form works

Clinical Treatment/Procedures

Results

[illegible]

Remarks
Pt confused (GCS 14: E4 V4 M6), not able to swallow safely, so no oral glucose. Glucose 2.1 at 1315. Glucagon 1 mg IM L deltoid at 1317. At 1324 still confused but talking more. At 1330 alert and oriented x4 (GCS 15) and able to describe what happened. Glucose 5.9. Pt ate a sandwich and drank 250 mL of juice at 1335 with husband present. Repeat glucose 6.8 at 1352. Recommended transport to ED because she received glucagon and her long-acting insulin is still working. Explained the risk that her glucose could drop again over the next several hours, including passing out while alone. Pt declined: "I know what happened, I skipped lunch. I'm not sitting in emerg for six hours." Pt repeated the risks back in her own words and gave her plan: husband home all afternoon and evening, supper at 1700, glucose checks every hour until bed and once overnight, call 911 if she is confused, sweaty, or her glucose is under 4. Aid to capacity assessment completed: all three questions yes. Advised to call her family doctor or diabetes clinic tomorrow. Husband present for the whole discussion and witnessed the refusal. Injection site checked, no bleeding. Left scene 1410.

Disposition of Effects
☐ Receiving Staff ☐ Family ☐ Other (list):

Primary Problem Diabetic Emergency (83)	Problem Code 83	Sp Trans Code	CTAS Arrive Patient 2
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Deceased ☐ Obviously dead ☐ DNR ☐ BHP TOR ☐ Pronounced by on scene physician				CTAS Depart Scene
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Physician/BHP Name (if pronounced/TOR)	Date	Time	CTAS Arrive Destination
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General Administration				
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Vehicle Number	Station	Status	Hospital Number	Receiving Facility/Destination
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1204	05	77							
UTM Code		Dispatch 4	Return 72	Patient 1	Sequence	Warning Systems	To Scene To Destination	☐ None ☐ None	☒ Emergency Systems ☐ Emergency Systems

Base Hospital Name	Base Hospital Number	Base Hospital Physician Name/No. (if patch)	Patch Log Number
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Call Events	Call Received 13:05:20	Crew Notified 13:05:50	Crew Mobile 13:07:10	Arrive Scene 13:12:30	Patient Contact 13:13:40	Depart Scene	Arrive Destination	TOC
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Paramedic 1 (Attending) No. 99001	Name Student, Alex	Designation 1	Signature No. 1 A. Student
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Paramedic 2 No. 12345	Name Preceptor, Sam	Designation 3P	Signature No. 2 S. Preceptor
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Other	Name	Designation	Signature No. 3
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Other	Name	Designation	Signature No. 4
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Date of ACR Completion 2026/09/19	Time of ACR Completion 14:40:00	1 – Patient Chart Copy 3 – Base Hospital Copy	2 – Billing Office Copy 4 – Ambulance Service Copy
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[Check my ACR](#)Read back

Clear highlights

Use when page 2 fills up. Rows continue from 27.

2026-0919-0412

Results

Crew	
br. No.	

Remarks

Remarks (continued)

[Check my ACR](#)

Read back

ACR Codes (1 of 2): Problem and Procedure Codes

Source: Ambulance Call Report codes, ontario.ca (page updated 2026/09/14). Codes change a few times a year: check ontario.ca/page/ambulance-call-report-codes.

Problem Codes			
VSA		116.01	Power Lifting Chair
1 Cardiac / Medical		117	Patient Lifting Device
2 Traumatic		117.01	Powered Lifting Device
Airway		231	Pt. Transported Supine
11 Obstruction (Partial/Complete)		232	Pt. Transported Semi-Prone
Breathing		233	Pt. Transported Prone
21 Dyspnea		234	Pt. Transported Semi-Sitting
24 Respiratory Arrest		235	Pt. Transported Sitting
Circulation		236	Ambulatory
31 Hemorrhage		237	Pt. Transported Lateral
32 Hypertension		238	Pt. Transported in Isolette
33 Hypotension		239	Infant Restraint Device
34 Suspected Sepsis		240	Knee Chest Position
Neurological		241	Exaggerated Sims Position
40 Traumatic Brain Injury		Suction and oxygen	
41 Stroke / TIA		120	Suction
42 Temp. Loss of Consciousness		120.1	Gastric Port Suctioning
43 Altered Level of Consciousness		129	Oxygen - Filtered High Conc. Mask
44 Headache		130	Oxygen - High Conc. Mask
45 Behaviour / Psychiatric		131	Oxygen - Simple Face Mask
45.01 Excited Delirium		132	Oxygen - Nasal Cannula
46 Active Seizure		133	Oxygen - Other
47 Paralysis / Spinal Trauma		141	Oxygen - BVM
48 Confusion / Disorientation		141.1	Manual Ventilation Feedback Device
49 Unconscious		142	Oxygen - Mechanical Ventilator
50 Post-ictal		144	Oxygen - Pocket Mask
Cardiac		Obstetrical	
51 Ischemic		150	Extricate Patient
53 Palpitations		160	OB Delivery
54 Pulmonary Edema		160.01	Delivery of Head
55 Post Arrest		160.02	Delivery of Umbilicus
56 Cardiogenic Shock		161	External Uterine Massage
57 STEMI		161.01	External Bimanual Compression
58 Hyperkalemia		162	Placental Delivery
Non-Traumatic		163	Clamp and Cut Umbilical Cord
60 Non Ischemic Chest Pain		164	Apgar Score
61 Abdominal / Pelvic / Perineal / Rectal Pain		165	Nuchal Cord
61.1 Renal Colic		165.01	Removal of Nuchal Cord
61.2 Suspected UTI		166	Shoulder Dystocia
62 Back Pain		166.01	McRoberts Maneuver
Gastrointestinal		166.02	Apply Suprapubic Pressure
63 Nausea / Vomiting / Diarrhea		166.03	Gaskin Maneuver
Integumentary		167	Breech Presentation
65 Integumentary		167.01	Manual Release of Limbs
65.1 Skin Tear		167.02	Mauriceau Smellie Veit Manoeuver
Musculoskeletal / Trauma		168	Prolapsed Cord Management
66 Musculoskeletal		Airway and breathing	
67 Trauma / Injury		170	Oro / Nasopharyngeal Airway
Obstetrical / Gynecological		180	Restrain Patient - Physical
71 Obstetrical Emergency		181	Spit Hood Application
72 Gynecological Emergency		190	Abdo / Chest / Back Thrusts
73 Newborn / Neonatal		211	Symptom Assist Medication
Endocrine / Toxicological		318	Supraglottic/Alternate Airway
81 Drug / Alcohol Overdose		319	Supraglottic/Alternate Airway Unsuccessful
81.1 Suspected Opioid Overdose		320	Needle Thoracostomy
81.2 Alcohol Intoxication		321	Needle Thoracostomy Unsuccessful
81.3 Drug Overdose		322	Needle / Surgical Cricothyroidotomy
82 Poisoning / Toxic Exposure		323	Needle / Surgical Cricothyroidotomy Unsuccessful
83 Diabetic Emergency		324	Nasotracheal Intubation
84 Allergic Reaction		325	Nasotracheal Intubation Unsuccessful
85 Anaphylaxis		326	Orotracheal Intubation
86 Adrenal Crisis		327	Orotracheal Intubation Unsuccessful
General and Minor		328	ETT Suctioning
87 Novel Medications		329	Tracheostomy Tube Suctioning
88 Home Medical Technology		330	Tracheostomy Tube Reinsertion
89 Lift Assist		330.01	Tracheostomy Tube Reinsertion Unsuccessful
90 Inter-facility Transfer		331	Magill Forceps / Foreign Body Removal
91 Environmental Emergency		332	Magill Forceps / Foreign Body Removal Unsuccessful
92 Weakness / Dizziness / Unwell		333	Extubation - Any Advanced Airway (Intentional)
93 Treatment / Diagnosis & Return		334	Extubation - Any Advanced Airway (Unintentional)
94 Convalescent / Invalid / Return Home		335	Needle Thoracostomy One-way Valve Monitored
95 Infectious Disease		336	Respiratory System Eval. (ETCO2 and SAO2)
96 Organ Retrieval / Transfer		337	ETT Confirmation
98 Organ Recipient		338	SpO2
99 Other Medical / Trauma (see remarks)		339	PEEP
99.15 No Complaint		380	Alternative Airway
Procedure Codes		381	Alternative Airway - Unsuccessful
Assessment		382	Airway Adjunct / Bougie
1 Clinical Note		383	CPAP
10 Vital Signs		384	CPAP - unsuccessful
20 Patient Assessment		385	Oro/Nasopharyngeal Swab
20.01 LAMS Assessment Score		386	Peak Flow Meter Testing
20.02 Acute Stroke Protocol Assessment		Cardiac	
20.03 Clinical Opiate Withdrawal Scale (COWS)		297	Therapeutic Hypothermia
20.04 Point-of-Care Risk Assessment		298	Defibrillation - Pads On
20.05 Clinical Frailty Scale		298.01	Pads Placement AL
20.06 NEWS2 Scale		298.02	Pads Placement AP
25 Blood Sampling - Glucose Determination		299	Automated CPR Device
Wound care		300	CPR
100 Dressing		301	Rhythm Interpretation
100.01 Hemostatic Dressing		302	Cardioversion
100.02 Occlusive Dressing		303	Valsalva Maneuver
100.03 Post-Op Dressing		305	Defibrillation Dual Sequential
100.04 Povidone Iodine Dressing		306	Defibrillation - Manual
100.05 Tissue Adhesive		307	Defibrillation - Semi-Automated
100.06 Reinforced Skin Closure Strip		308	Analyze - SAED
100.07 Fixation Sheet		309	External Pacing
100.08 Non-Adherent Dressing		313	12-Lead Acquisition
101 Control Bleeding		313.1	15-Lead Acquisition
102 Arterial Tourniquet		314	Cardiac Monitor Disconnected
Immobilization		316	Return of Spontaneous Circulation
105 Immobilization - Head		317	Return of Spontaneous Resp.
109 Reduction		IV and access	
109.01 Patellar Reduction		340	IV Monitoring
110 Splint Other		341	IV Cannulation
110.01 Pelvic Binder Device		342	Lock
111 Cervical Collar		345	Normal Saline
112 Spinal Board		349	Other IV Solutions
113 Spinal Immobilization Extrication Device			
114 Traction Splint			
Lifting and positioning			
115 Adjustable Break Away Stretcher			
116 Lifting Chair			

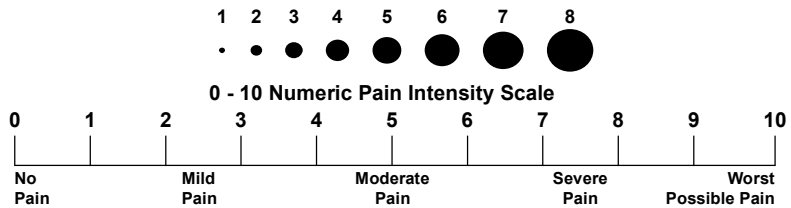
ACR Codes (2 of 2): Medications, Routes, Rhythms, Transport and Admin

Source: Ambulance Call Report codes, ontario.ca (page updated 2026/09/14). Codes change a few times a year: check ontario.ca/page/ambulance-call-report-codes.

Medication Codes		8	No STEMI Bypass - Pt. Condition	CTAS	Return priority can also be the CTAS level, 0 to 5
498	Acetaminophen	9	No STEMI Bypass - Hospital Refusal	No transport	
500	Adenosine	10	Palliative Care Pt. to Hospice/Hospital	8	Standby
502	Amiodarone	11	CMHA Middlesex Crisis Centre	9	Out of Service / Administration
503	Antibiotic	30	Specialized Team Transfer with Patient	71	No Patient Found
503.01	Amoxicillin	31	Specialized Team Transfer without Patient	72	Patient Refused
503.02	Cefuroxime	Trauma Site / Type / Mechanism			
503.03	Trimethoprim/Sulfamethoxazole	A Location			
503.04	Moxifloxacin	10	Head / Face / Ear / Scalp	73	Patient Deceased
503.05	Nitrofurantoin	11	Eye	74	Patient in Police Custody
503.06	Ceftriaxone	12	Neck	75	Transported by Other Ambulance
503.07	Cefazolin	13	Shoulder	77	Referred
503.08	Antibiotic - Ointment	14	Back / Flank	77.01	Referred - Palliative Care
503.09	Cephalexin	15	Chest	77.03	Referred - Mental Health and Addictions
504	ASA	16	Abdomen	77.04	Referred - Non-Emergent
505	Atropine	17	Pelvis	77.05	Referred - Non-Emergent LTC
525	Calcium Gluconate	18	Genitourinary	78	Discharge
528	Dextrose D10W	19	Buttocks / Perineum / Rectum	78.01	Discharged - Hypoglycemia
529	Dextrose D25W	20	Arm (Upper / Elbow / Forearm / Wrist)	78.02	Discharged - Seizure
530	Dextrose D50W	21	Hand / Finger	78.03	Discharged - Tachydysrhythmia
531	Diazepam	22	Thigh	78.04	Discharged - COVID-19
533	Dimenhydrinate	23	Leg (Knee / Lower Leg / Ankle)	78.05	Discharged - Mental Health and Addiction
534	Diphenhydramine	24	Foot / Toes	78.06	Discharged - Non-Emergent
536	Dopamine	25	Hip	78.07	Discharged - Non-Emergent LTC
537	Dexamethasone	Status Codes			
540	Epinephrine 1:1,000	0 At Base			
541	Epinephrine 1:10,000	22 Receiving Facility			
550	Fentanyl	44 On-Call			
551	Furosemide	77 Mobile			
560	Glucagon	88 Standby Location			
561	Glucose - Oral	99 Maintenance			
562	Hydroxocobalamin	Paramedic Designation			
565	Haloperidol	1 Student			
593	Lidocaine	2 EMA			
602	Ketamine	3 PCP			
603	Midazolam	4 ACP			
604	Morphine	5 CCP			
607	Hydromorphone				
610	Naloxone				
611	Buprenorphine/Naloxone				
615	Nitroglycerin				
620	Oxytocin				
650	Salbutamol				
651	Sodium Bicarbonate				
682	Xylometazoline				
700	Other Drugs - Detail in Procedures				
701	Anaesthetic Eye Drops				
704	Ibuprofen				
706	Ketorolac				
708	Obidoxime				
710	Pralidoxime Chloride				
711	Hydrocortisone				
712	Sodium Thiosulfate				
713	Glycopyrrolate				
728	Ondansetron				
729	Electrolyte - Potassium Elixir				
730	Lorazepam				
731	Immunization				
731.01	Tetanus				
732	Tranexamic Acid				
900-999	User Defined				
Routes of Administration					
AE	Aerosol	A	Airport / Heliport		
BU	Buccal	B	Apartment / Condo. Building		
ET	Endotracheal	C	Construction Site		
IH	Inhaled	D	Medical Office / Clinic		
IM	Intramuscular	E	Nursing Outpost		
IN	Intranasal	F	Factory / Industrial Site / Railway / Dockyard		
IO	Intraosseous	G	Hotel		
IV	Intravenous	H	Hospital (Acute and Non-Acute)		
NB	Nebulized	I	Indoor Shopping Mall		
PO	Oral	J	Jail / Prison		
PR	Rectal	K	Single Store / Strip Mall		
SC	Subcutaneous	L	School / College / University		
SL	Sublingual	M	Mining Site / Quarry		
TO	Topical	N	Long-Term Care Home		
Rhythm Codes		O	Office Building		
10	Sinus Tachycardia	P	Sports Facility / Arena		
11	PSVT / SVT / Atrial Tachycardia	Q	Farm		
12	Atrial Flutter	R	House / Town House		
13	Atrial Fibrillation	S	Street / Highway / Road		
14	Ventricular Tachycardia	T	Fairground / Park		
20	Sinus Bradycardia	U	Retirement Home		
21	First Degree Block	V	Golf Course		
22	Second Degree Block	W	Water / Boat		
23	Third Degree Block	X	Restaurant / Bar		
30	Ventricular Fibrillation	Y	Casino		
31	Pulseless Ventricular Tachycardia	Z	Other (Describe in Remarks)		
32	PEA	Za	Emergency / Transitional Shelter		
33	Asystole	Dispatch Priority			
40	NSR	1	Deferrable		
42	Paced Rhythm	2	Scheduled		
43	Junctional Rhythm	3	Prompt		
44	Sinus Dysrhythmia	4	Urgent		
46	Other (Detail in Procedures)	8	Standby		
CTAS		9	Out of Service / Administration		
0	Obviously Dead / TOR	P	MPDS Purple: emergent, immediately life-threatening		
1	Resuscitation	R	MPDS Red: emergent, potentially life-threatening		
2	Emergent	O	MPDS Orange: urgent, potentially serious		
3	Urgent	Y	MPDS Yellow: non-urgent, not serious		
4	Less Urgent	G	MPDS Green: non-urgent, may be deferred		
5	Non Urgent	SBY	Incident Standby		
Special Transport Codes		EMCOV	Emergency Coverage		
1	Pt. Meets Trauma Criteria	BSBY	Border Coverage		
2	No Trauma Bypass - Pt. Condition	Return Priority			
3	No Trauma Bypass - Hospital Refusal	1	Deferrable		
4	Pt. Meets Stroke Criteria	2	Scheduled		
5	No Stroke Bypass - Pt. Condition	3	Prompt		
6	No Stroke Bypass - Hospital Refusal	4	Urgent		
7	Pt. Meets STEMI Criteria	6	Transport of Deceased Patient		

Reference Information

Skin	
Colour	Condition
Flushed	Dry
Pale	Clammy
Cyanosis	Diaphoretic
Jaundice	Unremarkable
Unremarkable	



Glasgow Coma Scale		
Eye Opening	Verbal Response	Motor Response
4 Spontaneous	5 Orientated	6 Obeys commands
3 To Voice	4 Confused	5 Localize (pain)
2 To Pain	3 Inappropriate words	4 Withdraw (pain)
1 None	2 Incomprehensible sounds	3 Flexion (pain)
	1 None	2 Extension (pain)
		1 None

Pediatric Coma Scale		
Eye Opening	Verbal Response	Motor Response
4 Spontaneous	5 Coos or babbles	6 Obeys commands
3 To Speech	4 Irritable & constantly cries	5 Withdraws from touch
2 To Pain	3 Cries to pain	4 Withdraws from pain
1 None	2 Moans to pain	3 Flexion to pain
	1 None	2 Extension to pain
		1 None

APGAR Score			
Parameter	0	1	2
Appearance	Blue or Pale	Pink body with blue extremities	Completely pink
Pulse (BPM)	0 (absent)	slow (<100)	≥100
Grimace Response	None	Some grimace	Good grimace
Activity and Muscle Tone	None, limp	Some flexion	Active, motion
Respiratory Effort	absent	<60 min	Good, crying
APGAR performed at 1 & 5 minutes after delivery Don't wait for APGAR to make decision on resuscitation			

Normal Pediatric Vital Signs		
Age	Respiratory Rate	Heart Rate
0 – 3 months	30 - 60	90 - 180
3 – 6 months	30 - 60	80 - 160
6 – 12 months	25 - 45	80 - 140
1 – 3 years	20 - 30	75 - 130
6 years	16 - 24	70 - 110
10 years	14 - 20	60 - 90
Systolic Blood Pressure (for children 1-10 yrs) ≥ 90 + (2x age in years) Weight (kg) = (age x 2) + 10		

Definition of Hypoglycemia

≥2 years Glucometry <4.0 mmol/L
<2 years Glucometry <3.0 mmol/L

Aid to Capacity Evaluation (Record Details in 'Remarks' Section)

Indicate to whom this assessment refers if not the patient [e.g., parent, or substitute decision maker (SDM)]

The patient

Patient verbalizes/communicates understanding of clinical situation? (e.g., what is wrong with you?) ☒ Yes ☐ No - Requires consideration of capacity

Patient verbalizes/communicates appreciation of applicable risks? (e.g., what could happen if I don't help you?) ☒ Yes ☐ No - Requires consideration of capacity

Patient verbalizes/communicates ability to make alternative plan for care? (e.g., what will you do once I leave?) ☒ Yes ☐ No - Requires consideration of capacity

Responsible adult on scene ☒ Yes ☐ No

Refusal of Service – I have been advised that treatment and/or transportation is available immediately. I refuse such treatment and/or transportation to hospital having been informed of the risks involved. I assume full responsibility arising out of such refusal.

Refus de service – On m'a avisé que je pouvais être traité ou transporté à l'hôpital immédiatement. Je refuse d'être traité ou transporté à l'hôpital. J'ai été informé des risques auxquels cette décision m'expose. J'assume l'entière responsabilité de ce refus.

Patient/ Substitute decision maker (SDM) – print name and address / Patient/mandataire spécial (MS) – Nom et adresse en lettres moulées

Joanne Martel
17 Birch Crescent, Riverside ON

If SDM, relationship to Patient / Si MS, relation avec le patient

Time	Date	Signature of Patient or SDM / Signature du patient ou du MS
14:04	2026/09/19	J. Martel
I have advised this patient or SDM of the risks to the patient's health that are involved. J'ai avisé le patient ou le MS des risques de cette décision pour la santé du patient.		
Time	Date	Attending Paramedic Signature
14:05	2026/09/19	A. Student
I was witness to the above-mentioned refusal and that the person has been informed of the risks involved. J'ai été témoin du refus susmentionné et du fait que la personne a été informée des risques de ce refus.		
Time	Date	Non Paramedic Witness Name / Signature of Paramedic 2
14:05	2026/09/19	Paul Martel (husband) / P. Martel

Documentation Review

2026-0919-0412

Student fills section A before submitting. The checker fills section C. The instructor uses section D.

A. Before you submit (student)

What was the one decision on this call that your chart most needs to explain? Does it explain it?

Accepting the refusal. The chart shows capacity in her own words, the specific risks I explained, her plan, and who is staying with her.

If you ran this call again, what would you chart differently, and why?

I would chart the time she could swallow safely as its own line before she ate, so the order of events is clear.

B. Call summary (fills in from the chart)

Intervals

Crew notified to mobile: 1 min 20 s
Crew notified to arrive scene: 6 min 40 s
Arrive scene to patient contact: 1 min 10 s
On scene (arrive to depart): -
Transport (depart to arrive destination): -
Arrive destination to transfer of care: -
Patient contact to first 12-lead: no 12-lead charted
Patient contact to first medication: 4 min
Call received to transfer of care: -

Vitals trend

Sets of vitals: 4 (first 13:14, last 13:53)
HR: 108 > 100 > 92 > 86
BP: 146/86 > 140/84 > 138/82 > 134/80
RR: 18 > 16 > 16 > 16
SpO2: 98 > 98 > 98 > 98
GCS: 14 > 14 > 15 > 15
Pain: -
Shock index, first set (HR / systolic): 0.74

These numbers are for debrief, not grading. Interval targets depend on the call and your service. Ask what drove each one, and whether the chart would let someone else work it out.

C. Check my ACR results (filled by the checker)

Checked 14:39

No documentation gaps found by the checker. That doesn't mean the chart is good. It means nothing obvious is missing.

Questions the checker can't answer for you:

- Is there a glucose reading before treatment and a repeat after, both with times?
- Is level of consciousness described before and after treatment in specific terms?
- Does the chart show the patient could swallow safely before anything went by mouth?
- If they stayed home, does it record who is with them, whether they've eaten, and what they were told to watch for?
- Does the chart show, in the patient's words, that they understood their situation, the risks, and had a plan?
- Is it documented what you advised, including when to call back?
- Is your assessment before the refusal complete enough to support the decision?
- If the patient or a witness wouldn't sign, are the circumstances in Remarks? They don't have to sign (ODS 4.0).
- Did the responsible adult get instructions on what to watch for, follow-up, and possible complications, and is that on the chart (ACR Manual)?
- Does the Incident History say how and where you found the patient, and who told you what (ACR Manual)?
- Is your Primary Problem the most likely underlying cause, not the chief complaint restated? Shortness of breath can be the complaint and pulmonary edema the problem (ACR Manual).
- Could someone who wasn't on the call follow what happened from your Remarks alone?
- Is there something you decided not to do? The standards expect the reason for delaying or withholding an indicated treatment on the chart (ODS 4.0).
- If the patient declined any assessment or treatment, is it in the grid with their reason? If you had concerns the patient is at risk, are they in Remarks and in your handover (ODS 4.0)?

D. Instructor notes

How this practice ACR works

What this is

A practice version of the Ontario Ambulance Call Report (form 1881-45) for lab and simulation. It is not a patient record. It behaves like a careful paper chart, with a few extra tools that only work on screen and never print.

Use Adobe Acrobat Reader

The tools, checks and buttons need Acrobat Reader (free, desktop or iPad). In a browser or Mac Preview you can still type, but nothing will check or calculate. Save often.

The treatment grid

Vitals go in their columns, and so does a medication: the code, then the dose and unit, then the route. For a procedure, the ACR Manual has you write a short narrative in the columns next to the code: what was done and the result. Tick the small box in the left margin to do that. Everything from Dose/Unit to Pain Scale becomes one line, for example 341 then "20ga L forearm, successful and secured". A medication can be written across too, as long as the dose, unit and route are in the line: "160 mg PO, chewed and swallowed". Untick the box to go back to columns. If the narrative runs out of room, tick the next row and carry on. Page 3 is a continuation page with 26 more rows and extra Remarks. Page numbers update on their own.

Codes

Pages 4 and 5 list the current Ontario ACR codes, taken from ontario.ca (page updated 2026/09/14). Codes are written the way Ontario lists them: 10 for vital signs, and decimal sub-codes where they exist, such as 401.4 for a STEMI notification. The code list changes a few times a year, so check ontario.ca/page/ambulance-call-report-codes if something looks missing.

Checks as you type

Dates must be YYYY/MM/DD and times 24-hour (1952 becomes 19:52). Procedure codes are checked against the current code list, doses need a unit, and routes need a route code. Choosing a Primary Problem fills in the Problem Code. Ticking one option clears any option it contradicts.

Check my ACR

Finds documentation gaps: blank required fields, times out of order, procedures with no time or crew number, medications with no dose, route, or vitals before and after, CNO with no explanation, transfer of care with no receiving person, and places where the chart contradicts itself. Items marked (ODS 4.0) come from the Ontario Ambulance Documentation Standards. It highlights fields in yellow on screen and lists them, then asks questions it can't answer for you. The highlights clear before printing. A clean check means nothing obvious is missing. It doesn't mean the chart is good.

Read back

Builds a transfer of care report from what is on your chart. Say it out loud. If something you would want to tell the receiving nurse isn't in the read-back, it isn't in your chart.

Corrections

Once a chart is submitted, don't change the original entries. Add an addendum at the end of Remarks, clearly marked as an addendum, with the date and time you added it. This is how the Documentation Standards require corrections on a real call report.

Submitting

Fill in the reflection on the Documentation Review page, run Check my ACR, then press Lock for submission. Everything except the review pages becomes read-only and page 1 is stamped. Your instructor can unlock it.

Printing

Print pages 1 and 2, page 3 if you used it, and page 6 if there was a refusal. Tools, highlights and margin boxes don't print.

Where the rules come from

Checker items tagged (ODS 4.0) come from the Ontario Ambulance Documentation Standards. Items tagged (ACR Manual) come from the Ambulance Call Report Completion Manual, and most field tooltips quote its instructions for that field. Where the 2017 manual and the current code list disagree (for example special transport codes written 01 or 1, or the newer status codes), the form follows the current ontario.ca code list.

References

Ontario Ambulance Documentation Standards, version 4.0, effective 2025/09/02.

ontario.ca/document/patient-care-documents-reference/ontario-ambulance-documentation-standards

Ambulance Call Report codes, ontario.ca, page updated 2026/09/14. ontario.ca/page/ambulance-call-report-codes

Ambulance Call Report Completion Manual, version 3.0, in force 2017/04/01 (Ministry of Health and Long-Term Care). The field-by-field guide to the ACR.

Page map

- 1 Demographics, history and physical exam
- 2 Treatment grid, remarks, disposition and crew
- 3 Treatment continuation (only if needed)
- 4 Codes: problem and procedure
- 5 Codes: medications, routes, rhythms, transport and admin
- 6 Reference tables, capacity and refusal
- 7 Documentation Review (reflection, call summary, checker results, instructor notes)
- 8 This page